CONSULTATION: Evaluation of progressive bilateral visual field constriction and uncontrolled intraocular pressure.

HISTORY: A 71-year-old female presents with advanced primary open-angle glaucoma diagnosed 8 years ago. She currently uses latanoprost 0.005% at bedtime, brimonidine 0.2% BID, and brinzolamide 1% BID. She reports progressive difficulty navigating stairs, frequently bumping into door frames, and dark-adaptation delay.

OPHTHALMIC EXAMINATION:

Best-Corrected Visual Acuity (BCVA): Right eye 20/25; Left eye 20/40.

Intraocular Pressure (Goldmann Tonometry at 10:00 AM): OD 23 mmHg, OS 26 mmHg (target IOP <= 14 mmHg).

Central Corneal Thickness: OD 515 um, OS 510 um (thin corneas, underestimating true IOP).

Slit-Lamp Biomicroscopy: Bilateral clear corneas, quiet anterior chambers, deep and uninflamed. Normal irides without rubeosis, pseudoexfoliative material, or transillumination defects. Bilateral mild cortical and nuclear cataracts.

Gonioscopy (Gonio 4-mirror): Wide open bilateral angles to ciliary body band (Shaffer Grade 4) without synechiae.

Dilated Funduscopic Examination:

- Optic Disc OD: Severe glaucomatous cupping with cup-to-disc (C/D) ratio of 0.85, prominent inferior neuroretinal rim thinning, and a flame-shaped Drance hemorrhage at the 7 o'clock disc margin.

- Optic Disc OS: Near-total glaucomatous cupping with C/D ratio of 0.95, complete superior and inferior rim notch, and baring of circumlinear vessels.

- Macula & Periphery: Normal retinal architecture bilaterally without diabetic retinopathy.

HUMPHREY VISUAL FIELD (24-2 SITA-Fast):

- OD: Superior arcuate scotoma coalescing with an inferior nasal step; Mean Deviation (MD) -11.4 dB.

- OS: Dense double arcuate scotoma with tubular central 5-degree island of remaining vision; Mean Deviation (MD) -21.8 dB.

PLAN AND RECOMMENDATIONS:

1. Advanced, medically refractory primary open-angle glaucoma with progressive neuroretinal rim loss, active disc hemorrhage OD, and severe visual field loss.

2. Medical therapy has failed to achieve the target IOP of <= 14 mmHg.

3. Recommend surgical intervention: Proceed with Left Trabeculectomy with Mitomycin-C (0.2 mg/mL for 2 minutes) to arrest impending vision loss in the worse eye.

4. Schedule right eye for selective laser trabeculoplasty (SLT) vs trabeculectomy following recovery of the left eye.